MY DOCTOR
FAMILY PRACTICE

**EDITED: 23 March 2025**

## GENDER-AFFIRMING HEALTHCARE GUIDELINE

### Client information for feminising hormone therapy

Many transgender and gender diverse (TGD) clients choose to seek gender-affirming care in the form of hormone therapy. The decision to commence hormone therapy rests with you, the client, and not with your healthcare provider. The informed consent model of treatment gives you agency over this decision – it is the role of your doctor to support and guide you through this process safely and effectively.

You may have read up, or heard from other TGD persons, about hormone therapy. Some of the information that you may have come across could potentially be out of date or inaccurate.

If you have any questions or concerns at any time, then you should always feel free to raise these concerns with your healthcare provider.

Please remember that every client is unique and will respond differently to medication, and that one client's treatment programme might differ substantially from another's due to a variety of physiological and medical factors. Try not to compare the treatment that you are receiving with that of anyone else – your journey is your own. If you have concerns about the efficacy of your treatment, then make a point to discuss this with your provider at your next appointment.

Before commencing hormone therapy, there are a few points that are worth considering. This informed consent document will draw your attention to some of these points, as well as outlining the expected effects, side effects and risks associated with hormone therapy, to ensure that you have all the information you need to make the best decision about your body and your health.

### The role of endocrinologists and other specialists

Hormone therapy does not need to be prescribed or monitored by an endocrinologist. Prescribing hormone therapy is well within the realm of a suitably skilled general practitioner (GP) or family physician. Not all GPs, nor all endocrinologists, have experience in managing gender-affirming hormone therapy, and the expertise of the clinician should be the guiding factor in determining who prescribes and monitors your hormone therapy.

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You may benefit from seeing other allied health professionals, such as speech-language therapists, or from medical specialists such as plastic surgeons. Not every TGD client will necessarily want to pursue these options, and you should discuss your individual goals with your prescribing doctor.

### Home affairs and gender marker (South African Nationals ONLY)

If you are a South African Citizen and wish to update your gender marker on your birth certificate and identity document (ID), the Department of Home Affairs requires **two** letters from healthcare professionals which state that you have undergone medical or surgical gender reassignment. Either of these is sufficient; you do not need to have had surgery to update your gender marker.
Unfortunately, at present, gender markers in South Africa are binary; thus, an ID document can reflect either female or male. There is no unspecified or non-binary marker.

Your prescribing doctor can write one of your letters for the Department of Home Affairs and should be able to refer you to another healthcare provider to write the second letter.

If you wish to change your name, then this process should be undertaken separately (either before or after) to the process of updating your gender marker. This does not require supporting letters from healthcare providers.

**Potential challenges with legal documents:** Some TGD persons have trouble with banking, registration as a student and writing examinations, registration of motor vehicles, etc. while they are awaiting their new documents. Some are accused of potential fraud because they do not look the same as the photo in their ID or their name may be different from the name on their qualification certificates. To change one's gender marker and names will take a while. You may want to consider how you will deal with these challenges in the interim. You may also want to consider if you need to change other documentation, for example your matric certificate.

### Other important aspects to explore:

**Potential impact on emotions**: The impact of hormones can be very diverse and individualised. Your mood may fluctuate. For example, some TGD women may experience being more moody or tearful at times.

**Potential impact on relationships with family and significant others:** Have you thought about the possible impact on your relationship with significant others? If you are in an intimate relationship, this may change when you start on hormones, and relationship roles may need to be re-negotiated. A partner may grieve the loss of aspects of who you were and the way the relationship used to be. What possible impact can it have on your family and how will you be able to deal with your family's response? Have you considered the impact of the change of gender role in your family? Have you

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considered the impact of potential loss of fertility? Are there children that may be impacted and are the children prepared?

**Potential change in sexual orientation:** It is possible that your sexual orientation may remain constant or shift, either temporarily or permanently (e.g. shift in attraction or choice of sexual partners, widened spectrum of attraction, shift in sexual orientation identity)

**Potential impact on safety**: In some settings, the physical changes on hormones may affect your safety, with persons who do not fit into society's expectations of male or female being at increased risk of violence.

**Potential impact on employment**: Some TGD persons experience discrimination in the workplace, or struggle to obtain employment. This may be more difficult when your legal documents (e.g. ID) and your appearance do not match.

**Potential grief and loss:** Some TGD persons experience a sense of loss. A TGD man may lose certain gender roles in the family.

**Taste changes:** Some TGD persons experience a change in their taste sensations, and their likes and dislikes of certain foods.

**Body odour change:** Some TGD persons experience a change in their body odour on hormone therapy.

**Appetite and sleeping patterns:** Often the TGD person will experience an increase in appetite. This could cause weight gain. Sleeping patterns may also be affected.

## The role of psychotherapy in TGD clients

Seeing a psychologist is not a requirement for initiating hormone therapy. While previously, some healthcare providers required a letter of diagnosis or referral, this is no longer necessary under international best practice.

Your doctor will ask you some questions to determine that you have a good support structure as you move forward. This is not because taking hormone therapy itself necessitates this; rather, as many TGD clients experience, navigating the world is already difficult, and hormone therapy – although often resulting in many positive and beneficial changes – can also lead to some emotional lability. A solid support structure looks different for everyone, but may include friends, family, support groups, therapists and counsellors. Your doctor may suggest or offer you referrals to support groups and therapists, if you indicate that you may benefit from these.

## Feminising hormone therapy

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Feminising hormone therapy is prescribed for assigned male-at-birth clients who wish to feminise. The backbone of feminising hormone therapy is oestrogen therapy. Anti-androgens (spironolactone, cyproterone acetate or bicalutamide) are sometimes used, although many clients can achieve testosterone suppression using oestrogen therapy alone.

The biggest concern with oestrogen therapy is the risk of clot formation, which can lead to deep venous thrombosis (DVT) or life-threatening pulmonary emboli. It is this risk that limits the amount of oestrogen that healthcare practitioners can safely prescribe to clients. Oral oestrogen (Estrofem or Premarin, for example) carries a higher risk for these adverse events than parenteral (i.e. administered outside the digestive tract) oestrogen.

**Comparison of oestrogen preparations**

| Type | Examples | Advantages | Disadvantages |
|---|---|---|---|
| Tablet, taken orally | Estrofem<br>Premarin | • Easy to use<br>• Accessible at most pharmacies | • Higher thrombotic risk |
| Tablet, dissolved sublingually† | Estrofem | • Accessible at most pharmacies | • Time consuming (takes 30 minutes to absorb) |
| Injectable | Oestradiol valerate | • Safer<br>• Affordable<br>• Once-a-week dosing | • Available only from compounding pharmacy<br>• Requires knowledge of injection technique<br>• Requires disposables (syringes, needles, alcohol swabs) |
| Patch | Estradot | • Safer<br>• Available at most pharmacies<br>• Twice-a-week dosing | • Patches need to be applied and cared for correctly<br>• May cause skin reactions due to adhesive |

*†, Taking an oestrogen tablet sublingually involves holding the tablet in the mouth – either under the tongue, inside the cheek, or between the lips and teeth, while the tablet dissolves. After dissolving, the residue must be held in the mouth for a further 30 minutes to fully absorb, and any remnants need to then be spat out and the mouth thoroughly rinsed. It is important not to swallow at all during this time, as any swallowed medication will go through the digestive tract and carry a higher risk of adverse events.*

When taking **oral feminising hormone therapy**, oestrogen alone is often not enough to lower testosterone levels to the desired range. For this reason, an additional medication called a testosterone blocker (anti-androgen) is usually prescribed. These medicines help reduce the effects of testosterone, allowing the oestrogen to work more effectively in supporting feminising changes. Your doctor will monitor your hormone levels and adjust treatment as needed to ensure it is both safe and effective.

Your doctor will discuss with you the various options for hormone therapy that are available, and help you to decide which form of treatment is best for you.

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## Costs of hormone therapy

It is important to remember that not only do different clients have different needs in accessing hormone therapy, but also that prices for medication may vary between different pharmacies, and that these prices may fluctuate over time. The majority of clients can expect to spend approximately R600–R800 per month on hormone therapy. This does not include the cost of monitoring blood tests or doctor's visits.

| **Feminising Hormone therapy Cost (edited. 2026)** | | |
|---|---|---|
| **Initial Consultation with baseline bloods** | | |
| | **In-person consultation** | **Virtual consultation** |
| **Consultation** | R450 | R350 |
| **Baseline blood tests**<br>**Hb;HIV;Syphilis;Hepatitis;Creatinine;ALT; T.Chol (+ Glucose if virtual consult)** | +/-R325 | +/- R410 |
| **Estradiol Valerate (Fagron)** | R589.00 **every 3 months** ( + R125-R200 delivery fee) | |
| **Estrofem + Spironolactone** | +/- R 1200 **Per Month** (Dischem 23/01/2026) | |
| **Premarin + Spironolactone** | +/- R1100 **Per Month** (Dischem 23/01/2026) | |
| **Follow up Blood tests at 1 and/or 3 and 6 months on treatment** | | |
| **Monitoring blood tests (Hamadi Labs)**<br>**Hb; Creat; ALT; Total Estrogen; Testosterone** | +/- R340 | |
| **Virtual Consultation to review blood results** | R350 | |

*Pricing subject to change

## Changes that occur when using hormone therapy

The changes you will experience on hormone therapy often take some time to develop fully. Some of these changes are reversible and will disappear should you discontinue hormone therapy. Others are irreversible and will persist even if you stop taking your hormones.

The timeline for these changes to begin is variable, but most of them will only reach their maximum degree after 3–5 years on hormone therapy.

## Reversible changes

Such changes include:
• loss of muscle mass and decreased strength
• changes in body fat distribution, possibly associated with weight gain (increased fat deposition in breasts, buttocks, hips, and thighs)
• softer and thinner skin
• reduced acne
• lighter and thinner body and facial hair

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• cessation of male-pattern balding, possible scalp hair regrowth
• changes in sex drive (usually a decrease initially, followed by an increase together with a change in sexual response cycle)
• changes in strength and frequency of erections, and changes in the amount and consistency of ejaculate
• changes in mood and emotional response.

### Irreversible changes

Such changes include:
• breast development; while the size of breast tissue may fluctuate, hormone therapy will cause permanent development of breast structures that will remain even if hormone therapy is withdrawn
• testicular atrophy
• infertility
• changes in bone density.

### Limitations of hormone therapy

It is important to understand that there are certain features that hormone therapy cannot alter. Among others, these include:
• presence of facial hair – although hormone therapy may make the hair thinner, or cause it to grow more slowly, hormone therapy will not eliminate facial hair
• pitch of the voice
• bone structure of the face
• presence of thyroid cartilage (Adam's apple).

### Important risks associated with hormone therapy

As with any medication, hormone therapy carries with it certain risks. Some of these risks can be mitigated or reduced by lifestyle factors, while others are independent risks that cannot be altered. It is important to be fully aware of the risks associated with hormone therapy before making the decision to start your treatment.

Blood clots are the most prominent risk associated with feminising hormone therapy. A blood clot can lead to deep venous thrombosis (DVT), pulmonary embolism (a blood clot in the lungs), heart attack or strokes. These conditions may be severely debilitating or even fatal:
• cardiovascular disease
• nausea or vomiting
• migraines or other headaches
• gallstones and other diseases of the gallbladder
• elevated levels of prolactin can rarely occur in clients on feminising hormone therapy, due to the development of a prolactinoma, a benign (noncancerous) tumour of the pituitary gland, that may interfere with vision. These can require surgical management, depending on the nature of the lesion.

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Some of the risks mentioned are modified by other factors. Notably, cardiovascular and clot risk are worse in clients who:
• Are above the age of 45 years
• Smoke
• Use alcohol
• Have pre-existing medical conditions, such as diabetes, high blood pressure, or high cholesterol.

Some clients will experience a reduction in their blood pressure, and improvements in their cholesterol levels on hormone therapy. This is not a guarantee and is not a replacement for positive lifestyle changes

## Fertility

Although not all clients become infertile on hormone therapy, and some might regain fertility if they stop hormone therapy, many may become irreversibly infertile. Hormone therapy is not a replacement for effective and responsible contraceptive use.

All clients considering starting on hormone therapy should consider using a cryobank to preserve genetic material, in case they wish to conceive genetically related children at a later stage. Even if this is not a priority for you at this stage in your life, please consider the possibility that your perspectives might change with time, and that it is ideal to store material before starting hormone therapy, rather than trying to regain fertility once you are already taking hormones.

**Fertility Preservation for Trans Women**

Sperm Freezing (Cryopreservation) – Cape Town: Allows you to preserve the option of having biological children in the future.

Sperm freezing (also called cryopreservation) involves collecting, freezing, and safely storing sperm for future use. Stored sperm can be kept for many years.

Oestrogen and testosterone blockers can reduce or stop sperm production, sometimes permanently. Freezing sperm before starting hormones keeps future family-building options open. You do not need to know yet whether you want children.

Trans women and transfeminine people or anyone producing sperm who wants to preserve fertility before hormone therapy or surgery can use this service.

The process involves one or two clinic visits; Private sperm sample collection; Testing, freezing, and secure storage; Annual storage with the option to continue long-term. The process is safe, confidential, and respectful.

Costs (approximate – Cape Town)

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Prices vary by clinic
Initial freezing + first year storage: ± R3,500 – R7,000
Annual storage thereafter: Clinic-specific (you will be informed upfront)

Sperm freezing is offered through fertility clinics in Cape Town, including:
Cape Fertility
HART Fertility Clinic
Aevitas Fertility Clinic
Feel free to contact any of these facilities for more information related to their specific service.

**Important to know**
Freezing sperm does not significantly delay starting hormone therapy
Hormones do not reliably restore fertility once sperm production stops
Stored sperm can later be used with IVF or other fertility treatments
This is a personal choice, there is no pressure to proceed.

## Monitoring and follow up

Your doctor will advise and guide you in monitoring your safety while you are on hormone therapy. Usually, this will involve regular check-ups and physical examinations, as well as certain blood tests.

At the outset, it is not uncommon for these evaluations to be done monthly, while you are still achieving the correct hormonal balance for you. Later, once you are stable on treatment, these intervals might be extended to six-monthly, or perhaps even annually. This schedule is different for every client.

## Cessation of hormone therapy

If you decide to stop your hormone therapy, **you should discuss this decision with your doctor.** It can be dangerous to abruptly withdraw hormone therapy without adequate medical supervision.

## More information

Please remember that you can discuss any questions or concerns with your doctor at any time.

Information on self-injection technique can be found at:
**Youtube:**

1) **How to perform a subcutaneous injection - gender affirming hormone therapy - Dr Anastacia Tomson**

   https://www.youtube.com/watch?v=1uehXqNh8p4

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